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Understanding Facet-Mediated Neck Pain: Patient Information Leaflet

This leaflet provides information about a common cause of neck pain and headaches. It aims to help you understand your condition, its treatment options, and how it differs from other types of neck and head pain.

What are Facet Joints?

Your spine is made up of bones called vertebrae. At the back of each vertebra are two small joints on each side called facet joints. Think of them like the hinges of a door, guiding and controlling the movement of your spine. These joints are lined with cartilage and surrounded by a capsule, and like any other joint in your body, they can become painful due to wear and tear, injury, or inflammation. This is known as facet joint syndrome or facet-mediated pain.

What Does Facet-Mediated Neck Pain Feel Like?

Facet joint pain is typically a deep, aching pain worse with movement. It is often felt directly in the neck but can also be referred to other areas, including top of the shoulders, upper back between shoulder blades and back of the head causing occipital headaches as detailed below.

Primary Location

You may feel pain and stiffness in the neck, often worse in the morning or after periods of inactivity.

Pattern of Referral

The pain commonly spreads (refers) to your shoulder, shoulder blade, and the back of your head. It rarely goes past the elbow or causes tingling in the hands.

What Makes it Worse

Movements like looking over your shoulder, bending your neck backwards (drinking out of a champagne glass), or holding your head in one position for a long time (e.g. working at a computer) can aggravate the pain.

Cervicogenic Headaches (Headaches from the Neck)

A very common symptom of facet-mediated neck pain is a cervicogenic headache. This is literally a "headache that originates from the neck."

What it feels like

A dull, aching pain that is typically felt at the back of the head, but can also wrap around to the forehead, temples, and behind the eyes.

The Link to Your Neck

The pain is referred from the upper cervical facet joints (C1-C3) and surrounding structures. It is often triggered or worsened by certain neck movements or sustained postures.

Key Difference

Unlike migraines or tension headaches, a cervicogenic headache is directly caused by a problem in the neck. You will often have neck pain, stiffness, or tenderness at the same time.

Whiplash and Facet Joint Pain

Whiplash is a common cause of facet-mediated neck pain and cervicogenic headaches. During a whiplash injury (like a car accident), your neck is suddenly jerked forward and backward. This rapid movement can injure the facet joints, tearing their capsules and causing inflammation and pain that can become chronic, often leading to persistent headaches.

How is it Different from Radicular Pain (Pinched Nerve) in the neck?

It is important to distinguish facet pain from radicular (nerve) pain, as they are treated differently.

- Pinched nerve pain is sharp, shooting, electric-like. It can be in the neck, shoulder, shoulder blade, back of head or arm and hand. Pain travels from the neck down the arm in a specific pattern (dermatome).
- Other symptoms include local tenderness, muscle spasm, numbness, tingling, pins & needles in the arm or hand. Muscle weakness is not typical but can happen in the arm or hand.
- Typically caused by a disc herniation or bone spur pressing on a nerve root due to degenerative changes in the vertebrae. Sometimes inflammation of the facet joints can irritate the nerve and cause a similar pain.
- In short, facet pain is an ache that refers to the shoulder, upper back and head, often causing cervicogenic headaches. Radicular pain is a sharp shot that travels down the arm and is associated with tingling or weakness.

Treatment Pathway for Facet-Mediated Pain

Treatment usually starts with conservative measures like physiotherapy, anti-inflammatory medication, and activity modification. If these do not provide sufficient relief, your doctor may recommend a series of targeted injections.

Step 1: Diagnostic & Therapeutic Injections

The goal is first to confirm the facet joint as the source of pain and to provide relief.

Medial Branch Blocks (MBBs):

- The small nerves that carry pain signals from your facet joints are called medial branch nerves. The nerve at C2-3 Level is called Third Occipital Nerve (TON) and commonly caused headache.
- In this procedure, a doctor uses ultrasound or x-ray guidance to place a small amount of local anesthetic (numbing medicine) onto these specific nerves.
- This is a diagnostic test. If your pain (including your headache and shoulder pain) is significantly reduced (e.g., 80% or more) for the duration of the anesthetic, it confirms that your pain is coming from the facet joints. The relief is temporary (a few hours).

Facet Joint Cortisone Injections:

As a first step, commonly an anti-inflammatory steroid (cortisone) is injected directly into the facet joint.

This aims to reduce inflammation within the joint itself, providing longer-lasting pain relief (weeks to months), which can also alleviate associated cervicogenic headaches, shoulder and upper back pain.

Step 2: Radiofrequency Ablation (RFA) / Neurotomy

If you get good, but temporary, pain relief from a medial branch block and the cortisone gives you just a short lived pain relief, you may be a candidate for a longer-term solution.

What is Radiofrequency Ablation?

It is a procedure that uses heat energy to temporarily "disable" the TON and medial branch nerves, stopping them from sending pain signals to your brain.

How does it work?

Using precise x-ray or ultrasound guidance, a special needle is placed next to the target nerve. A mild electrical current is then used to create a heat lesion on the nerve.

What to expect?

The first 3-4 weeks after a cervical RFA are tough and pain is aggravated at times. Once pain relief settles in you could expect 6 to 12 months of pain relief. The procedure meaningfully reduces both neck pain and cervicogenic headaches. This allows you to participate fully in physiotherapy and daily activities without pain. At times pain relief could last less than 6 months. The pain relief from RFA is not permanent, as the nerves regenerate. The procedure can be repeated if the pain returns.

There is a separate leaflet available for further details on RFA.

Summary

- Facet joints are common sources of neck pain and cervicogenic headaches, especially after injuries like whiplash.
- The pain is a deep ache, often felt in the neck, shoulders, and back of the head.
- Generally it is different from radicular pain, which shoots down the arm and causes tingling, but at times can be similar.
- Treatment can progress from physiotherapy to diagnostic injections (medial branch blocks) and, if successful, to a longer-lasting treatment (Radiofrequency Ablation) that can relieve both neck pain and headaches.

Always discuss your symptoms and treatment options thoroughly with your doctor or pain specialist to determine the best plan for you.

Questions? Contact Your Care Team

Before/after procedure:

 782-377-7813

 Info@redsandspm.com

In an emergency

→ Call 911, or visit your local emergency department

Disclaimer

This leaflet is for informational purposes only and does not replace professional medical advice. Diagnosis and treatment should be made by a qualified healthcare provider.